



In shelters across Lebanon, UNICEF provides displaced children and families with safe spaces for learning, play, and recreation.

Highlights

- Escalating hostilities in Lebanon have reportedly killed at least 1,318 people, including 125 children, and displaced over 1.1 million people, including more than 390,000 children, overwhelming systems and significantly increasing risks for affected populations.
- UNICEF is scaling up its emergency response, which has included sustaining access to safe water and sanitation for over 2.6 million people, including 385,000 displaced people, through the provision of fuel and critical repairs to water and wastewater systems.
- UNICEF's rapid response mechanism has reached 188,155 people in shelters, on the move, and in hard-to-reach areas with essential items.
- To reach 1 million people in need, UNICEF requires US\$ 48.3 million for its three-month response, but faces a 77 per cent funding gap with only US\$ 10.9 million available. Without urgent additional funding, life-saving services for children and families will be disrupted.

Lebanon

Humanitarian Flash Update No. 6

Escalation of Hostilities

2 April 2026

unicef 
for every child

Situation in Numbers



390,000
Children displaced



1.1 million
People displaced



125
Children killed since 2 March
(MoPH)



429
Children injured since 2 March
(MoPH)

Situation Overview and Humanitarian Needs

Hostilities in Lebanon continue to have a devastating impact on civilians. As of 1 April, at least 1,318 people have reportedly been killed, including 125 children, and 3,935 injured, including 429 children, according to the Ministry of Public Health (MoPH).

Mass displacement is ongoing amid intensified strikes, repeated displacement orders, and the expansion of affected areas. As of 1 April, over 1.1 million people¹ are displaced, including more than 390,000 children, with 136,321 individuals sheltering in 669 official collective shelters.² Continued strikes in densely populated urban areas, including Beirut's southern suburbs, risk triggering further large-scale displacement and compounding already overstretched shelter capacity.³

Cross-border movements into Syria continue to rise; between 2 and 27 March, over 202,477 individuals crossed from Lebanon into Syria through official border points, including 175,134 Syrians and 27,343 Lebanese nationals.⁴

¹ Ministry of Social Affairs

² Disaster Risk Management Unit Daily Situation Report, 1 April

³ OCHA Lebanon: Flash Update #13 - Escalation of Hostilities in Lebanon as of 30 March 2026

⁴ UNHCR Syria: Flash Update – Middle East Situation, 29 March 2026

Hostilities remain widespread across Beirut's southern suburbs, southern Lebanon, and the Bekaa, with continued artillery fire and reported ground incursions along the Blue Line.⁵ Civilian infrastructure continues to be impacted, including 17 reported incidents affecting water systems across Baalbek-Hermel, Bekaa, South, and Nabatieh, damaging reservoirs, pumping stations, solar systems, and main supply lines, and disrupting access to safe water.

Attacks on healthcare and emergency services remain a major concern. Since 2 March, 53 healthcare workers have been killed and 137 injured, alongside 82 reported attacks on emergency medical services. A total of 67 vehicles and 20 medical facilities have been impacted, and five hospitals have been forced to close.⁶ Overcrowded shelter conditions are contributing to the emergence of communicable diseases, including scabies, lice, and varicella, increasing public health risks. Access to health services remains constrained, particularly for populations in hard-to-reach areas with limited mobility. These challenges are compounded by the impact of ongoing hostilities on healthcare facilities, further reducing the availability and accessibility of essential services.

Education in Lebanon has been severely disrupted by escalating hostilities and mass displacement. Of the 669 collective shelters, 364 are public schools and 58 are Technical and Vocational Education and Training (TVET) institutions, greatly reducing available learning spaces. In addition, 439 public schools across South and Nabatieh, Bekaa, Baalbek, and Beirut's southern suburbs remain closed due to insecurity, evacuation orders, and proximity to conflict zones. These closures have directly affected approximately 256,000 students, including 181,500 in the morning shift and 74,500 in the afternoon shift. The conversion of 58 TVET schools into shelters has disrupted access for 16,400 youth, while the closure of 46 additional TVET schools in South Lebanon and Bekaa has affected a further 16,850 youth. Following three weeks of school closures, the Ministry of Education and Higher Education (MEHE) has initiated a gradual reopening of schools through blended learning modalities. Around 300 schools have resumed learning in safer locations, primarily in North Lebanon and Akkar, including 50 designated hub schools operating double-shift systems to accommodate students from closed or shelter schools. Further reopenings in Mount Lebanon and Bekaa are anticipated. At the TVET level, 55 public schools have resumed in-person learning in safe areas, while 103 schools, either closed or currently used as shelters, have transitioned to online learning delivered by TVET teachers. However, despite these efforts, access to education remains uneven, particularly for displaced children and youth facing barriers such as limited connectivity, lack of safe learning spaces, and ongoing insecurity, constraining continuity of learning. Prolonged disruption to education risks increased dropout rates and negative coping mechanisms among children and adolescents, particularly among displaced populations.

According to a recent rapid gender analysis by UN Women, protection risks in Lebanon are increasingly driven by the conditions of displacement. Overcrowding, lack of privacy, insecure living arrangements, and dependence on others for shelter are heightening exposure to gender-based violence, exploitation, and domestic tension. At the same time, mobility restrictions, information gaps, and documentation barriers are limiting access to humanitarian assistance, disproportionately affecting women, particularly those with caregiving responsibilities. Displacement is also increasing unpaid care burdens and reducing access to income and resources, while households adopt harmful coping strategies, further straining their capacity. Risks are particularly acute for female-headed households, adolescent girls, older women, persons with disabilities, and migrant and refugee women. These dynamics underscore the need for targeted, gender-responsive interventions to ensure safe and equitable access to services.⁷

Summary Analysis of Programme Response

Water, sanitation and hygiene

In coordination with the Ministry of Energy and Water (MoEW), UNICEF supplied over 280,000 litres of fuel to Water Establishments, supporting the operation of approximately 45 water facilities, including water pumping stations, and 19 wastewater stations nationwide. In parallel, UNICEF undertook critical repair and maintenance works, including 42 repairs to water systems, primarily water pumping stations and associated networks, and six repairs to wastewater systems. These fuel, repair, and maintenance interventions directly supported the continuity of public water supply and wastewater services for more than 2,650,000 people, including an estimated 385,000 IDPs hosted within communities relying on these systems.

UNICEF partners are delivering emergency WASH assistance in 216 collective shelters across Lebanon, reaching an estimated 42,822 IDPs, alongside the distribution of essential hygiene items through the Rapid Response Mechanism (RRM). Support in collective shelters includes repairs to existing WASH facilities and the installation of new showers and latrines,

⁵ OCHA Lebanon: Flash Update #13 - Escalation of Hostilities in Lebanon as of 30 March 2026

⁶ Public Health Emergency Operation Centre Update – 1 April 2026

⁷ Rapid Gender Analysis - Gendered Impacts of Displacement Outside Collective Shelters in Lebanon, UN Women, March 2026

including disability-friendly units. In addition, 187 centre cleaning kits were distributed to support safe and hygienic shelter operations.

Furthermore, 7,142 cubic metres of water was provided through water trucking, reaching 18,772 IDPs, while 295 cubic metres of sludge was safely removed through desludging operations in shelters hosting displaced populations. Rapid WASH technical assessments have been completed in 98 per cent of shelters to identify evolving needs, including urgent diesel requirements to sustain critical water and wastewater pumping stations. Displacement locations are being mapped to assess connectivity to existing water networks and determine the need for water trucking or alternative supply options. Wastewater pumping stations continue to be closely monitored to prevent overflows and mitigate associated public health risks.

Rapid Response Mechanism

UNICEF's RRM has been activated since 2 March 2026 to address displacement waves and reach families on the move, in shelters, and those remaining in hard-to-reach areas, serving as an agile, multi sectoral response platform linked to WASH, nutrition, protection, and other emergency interventions.

As of 1 April, UNICEF's RRM has reached 188,155 IDPs in more than 360 shelters, people on the move, and in hard-to-reach areas with essential non-food items (NFIs). Distributions included 446,319 bottles of water (1.5L each), 30,823 family hygiene kits, 15,416 baby kits, 19,188 dignity kits, 1,882 packs of adult diapers and sanitary pads, and 1,330 boxes of high-energy biscuits for children aged six months and above and pregnant women. Winterization support included 28,466 blankets and 21,301 children's clothing kits. In addition, around 4,700 families on the move in Beirut, Saida, and Tyre were supported with light RRM kits and bottled water.

As part of the RRM response, joint WFP–UNICEF convoys continued to deliver life-saving supplies to children and families in hard-to-reach areas, despite severe access constraints and ongoing hostilities. Convoys reached approximately 27,000–28,000 people, including 9,000–10,000 children, across Tyre, Rmeich/Ein Ebel/Debel, and Qlaya'a–Marjaayoun, delivering safe water, hygiene kits, family hygiene kits, dignity kits, and baby kits. UNICEF continues to prioritise access to hard-to-reach areas, sustaining delivery of life-saving support while assessing gaps to inform a more continuous, child-centred response. In parallel, a UNICEF-led mission to Nabatieh on 31 March delivered 2,200 hygiene kits to support 4,075 families, including IDPs, across 26 villages in the Al Sheqif Union of Municipalities. This intervention targeted both displaced and host communities living in areas witnessing active fighting, ensuring coverage across collective centres and villages hosting IDPs.

Child protection and GBViE

Community-Based Psychosocial Support (CBPSS) reached 8,675 children and caregivers through in-person sessions in shelters and community spaces, combining structured support with recreational activities to promote well-being, resilience, and positive social interaction. Caregivers were also supported with practical guidance on stress management, communication with children, protection risks, and available services and referral pathways.

A total of 26 children at risk or survivors of violence received case management services. In parallel, 21 unaccompanied and separated children were identified, of whom 20 were reunified or placed in community-based care. Awareness materials on child separation during conflict have been disseminated across 40 public and private hospitals nationwide.

Child protection and psychosocial support messaging reached 16,167 individuals through direct outreach and social media, promoting safer practices and strengthening families' ability to support children. The digital explosive ordnance risk education (EORE) campaign, launched across multiple platforms, has reached over one million individuals nationwide, including in hard-to-reach areas. In addition, 473 frontline workers were reached through online sensitization sessions, and 31,966 EORE IEC materials have been distributed in shelters across the South, West Bekaa, and Baalbek-Hermel.

Efforts to mitigate heightened gender-based violence (GBV) risks continue to be scaled up, including awareness sessions on GBV and sexual exploitation and abuse (SEA), psychosocial activities, and GBV safety audits in collective shelters. Dignity and LAHA kits have also been distributed in shelters. PSEA materials, including reporting procedures and hotlines, have been displayed across shelters, while POWER4Girls emergency sessions continue to provide adolescent girls with life-saving information and practical skills. Despite these efforts, identification of GBV survivors remains limited due to overcrowding, lack of privacy, and non-conducive conditions in collective shelters, which constrain safe disclosure. Ongoing awareness-raising and outreach aim to improve reporting and access to services.

Education

UNICEF continues to support MEHE in rolling out a phased response to ensure continuity of learning for children affected by the crisis. After more than three weeks of school closures, MEHE has begun gradual reopening in safe areas, combining in-person and online learning. Around 300 schools, mainly in North Lebanon and Akkar, have resumed learning, supporting 116,800 students through the Transition and Resilience Education Fund (TREF). This includes 50 hub schools hosting students from other closed or shelter schools on alternating schedules. Further reopenings are expected in Mount Lebanon and Bekaa.

UNICEF is also supporting universal remote learning for all public school students through multiple digital platforms. Accounts and virtual classrooms have been established, reaching 40,000 teachers and 390,000 students. Collaboration with the Ministry of Telecommunications provides up to 20GB of free data per weekday, while monitoring mechanisms on MS Teams and Madristi track participation. In the first two weeks, 56,000 students and 12,400 teachers logged in, highlighting uptake of remote modalities.

Through the Multiple Flexible Pathways (MFP) programme, 607 education kits have been distributed in 104 collective shelters, reaching 30,350 children with items such as learning supplies, recreational supplies, and LEGO playboxes. In addition, 26,227 children are accessing integrated education services across 32 Makani centres and 33 Dirasa schools in safe locations. Community outreach efforts by implementing partners have enabled 900 displaced children from nearby shelters and host communities, particularly in Beirut and Mount Lebanon, to participate in Makani-based learning and integrated support activities, with continued work to scale up capacity in all existing and open Makani centres and Dirasa schools in proximity to shelters and catchment areas of displaced children, as well as introducing additional shifts and a rotational schedule to maximise access to learning and integrated services for crisis affected children.

The work to establish Temporary Learning Spaces (TLS) to support displaced and crisis-affected students is ongoing. To date, 30 out of the 75 targeted TLS have signed agreements to host IDP students across Bekaa, Baalbek, Saida, Mount Lebanon, and Beirut, with a phased rollout of activities underway. These TLS will deliver a range of flexible, context-adapted education modalities, including Retention Support (RS) for in-school children and Accelerated Learning Programme for out-of-school children, complemented by psychosocial support, recreational, and sports activities. In addition to in-person learning, the approach will integrate digital and hybrid learning options, including the use of digital hubs, to ensure continuity of learning for displaced children where access to physical spaces remains limited.

The Inclusive Education Programme is mobilizing MEHE multidisciplinary teams to support children with disabilities during the current crisis. To ensure seamless support across online, blended, and in-person modalities, approximately 315 paraprofessionals and special educators training were trained on the Microsoft Teams and Madristi platforms on 26 and 27 March. Updated terms of reference were issued last week to reflect the specific roles and responsibilities in emergency learning contexts. Support for children with disabilities is also being integrated into MFP activities, including the provision of assistive devices, therapy, and referrals. In shelters, to date, MFP partners have identified 273 children with disabilities and 204 adults with disabilities. Of these, 45 children with disabilities have received assistive devices, 45 have started therapy services, and a total of 52 children with disabilities have been supported so far. In addition, 75 adults with disabilities have been referred to other service providers. This multi-pronged approach ensures continuity of education, inclusion, and protection for all learners, particularly displaced children and those with disabilities.

Health

Since the onset of the emergency, UNICEF has supported the health response for displaced populations through the delivery of more than 140 tons of essential medications and medical supplies to MoPH Primary Healthcare Satellite Units (PSUs) and governmental hospitals. UNICEF has supported the MoPH to activate mobile health services through its PSU and AaSalamah interventions, delivering primary healthcare services to displaced populations. These clinics cover up to 30 per cent of the total shelters covered by the MoPH, providing essential primary healthcare services. In addition to internally displaced persons (IDPs) residing in shelters, UNICEF-supported units cover host communities and hard-to-reach areas. Currently, 18 PSUs and 16 AaSalamah centres remain operational in hard-to-reach locations. To date, PSUs (mobile clinics) have delivered more than 23,717 primary healthcare services, with 10,627 beneficiaries receiving essential medications.

Health surveillance in shelters indicates that the most common conditions identified are communicable conditions, particularly lice and scabies, linked to overcrowding and living conditions. In response, UNICEF has supported the MoPH with large quantities of treatment supplies for both conditions, which have been directly distributed to all PSUs and AaSalamah

centres since the beginning of the crisis. These treatments continue to be delivered to patients as part of routine service provision, in response to the ongoing spread of these conditions in shelters.

Through the AaSalameh primary healthcare community outreach programme, 5,006 children were vaccinated to date. UNICEF also continues to sustain support to the MoPH Public Health Emergency Operations Centre (PHEOC) to ensure responsiveness and to central medication warehouses, strengthening coordination, logistics, and staffing to ensure the uninterrupted supply, proper storage, and distribution of essential medications to PHC centres and governmental hospitals.

Critical care needs are also being addressed, with 156 newborns and children admitted to intensive care units across 11 governmental hospitals. Under the UNICEF–MoPH Assistance and Care for War-Wounded and Affected Children (ACWA) programme, 20 children with conflict-related injuries received life-saving surgical interventions, while 75 injured children and caregivers received mental health and psychosocial support.

Nutrition

UNICEF has identified a critical gap in the availability of breastmilk substitute (BMS) kits for non-breastfed infants aged 0–12 months residing in shelters. These kits are essential to ensure the safe preparation and use of infant formula when breastfeeding is not possible and only following appropriate assessment. In response, UNICEF, in its capacity as Nutrition Sector lead, has advocated for the prioritization of BMS kits among partners and issued a Letter of Invitation to Bid for the procurement of 3,000 kits.

A total of 2,080 caregivers of children under five received integrated Infant and Young Child Feeding (IYCF), nutrition, and Early Childhood Development (ECD) counselling. Of these, 375 caregivers accessed guidance on optimal infant feeding practices in emergencies through the IYCF chatbot, while 207 caregivers contacted the national IYCF hotline, with 134 referred for specialized support. In parallel, IYCF rapid assessments reached 96 caregivers to identify and refer children requiring specialized services.

UNICEF continues to integrate essential nutrition and ECD interventions across multiple service delivery platforms, including Primary Health Care Centres (PHCCs), PSUs, nutrition partners, and education partners, to support internally displaced and vulnerable populations. To date, 40 PSUs have been supported with nutritionists, extending coverage to more than 99 shelters. Of these, 37 PSUs are actively delivering nutrition services across 87 shelters, while three additional PSUs have recently become operational following recruitment and capacity building on nutrition and wasting management.

Eight education partners have been mobilized to deliver ECD-in-emergencies services across more than 53 collective shelters. Integrated nutrition and ECD services also continue through Makani centres in host communities, ensuring continuity of care and expanded coverage beyond shelter settings.

To prevent wasting among high-risk groups, 240,500 high-energy biscuits (HEBs), 167,412 ready-to-use complementary food (RUCF) jars, and other energy-protein supplements have been prepositioned, sufficient to support over 30,074 children and pregnant and breastfeeding women (PBW) for one month.

To date, 9,804 children under five have received emergency food rations and micronutrient supplementation sufficient for one month, including HEBs, small-quantity lipid-based nutrient supplements (SQLNS), and complementary food jars. In addition, 624 pregnant and breastfeeding women and adolescent girls received micronutrient supplementation to prevent deficiencies. Screening efforts reached 4,166 children under five and 534 PBW, identifying 92 children and 25 women with wasting, all of whom were referred for treatment.

Social Protection and Humanitarian Cash Assistance

As part of the immediate response to the current escalation and to support beneficiaries of the National Disability Allowance (NDA), UNICEF and the Ministry of Social Affairs (MoSA) advanced the regular cash assistance payment for 38,000 Lebanese and non-Lebanese NDA beneficiaries nationwide, beginning on 10 March.

Building on previous emergency cash responses in 2024, UNICEF, MoSA, and the ILO have activated emergency financial support for families with persons with disabilities affected by the conflict. This shock-responsive expansion of the NDA has provided urgent cash assistance to offset additional costs related to disability during displacement, including access to healthcare, assistive services, and basic needs. Using established NDA delivery systems and registries, the first phase of this intervention was launched on 11 March and has reached approximately 8,000 families (6,000 Lebanese and 2,000 non-

Lebanese, or around 33,000 individuals) as of 19 March, with each household receiving a US\$100 grant for households with a child or young person aged 0–19 years living with a disability.

A second phase of the shock-responsive expansion of the NDA has been initiated, through an additional allocation of resources. In this second phase, an additional 4,000 families (3,000 Lebanese and 100 non-Lebanese, or around 16,000 individuals) with a young person aged 20–27 years living with a disability will be receiving, starting 2 April, a US\$ 100 emergency grant.

The overall target for this emergency support is 45,000 families with a person with a disability from affected areas, to be reached in a phased approach over three months through a one-off cash transfer based on the availability of resources. This programme complements the national Shock-Responsive Social Protection system, also led by the government, ensuring continuity of support and mitigating the immediate financial impact of displacement on vulnerable households.

Youth and Adolescent Development

UNICEF continues to engage youth as active contributors to emergency response efforts, supporting both service delivery and community resilience.

To ensure continuity of formal TVET education, UNICEF provided technical support to the TVET Directorate to finalize and implement its learning continuity plan, in line with the Government's directive to resume learning from 30 March 2026. The plan includes both online and in-person modalities, targeting 51,600 TVET students and youth across Lebanon. To date, 55 public TVET schools have progressively resumed in-person learning in safe areas, while 103 schools, either closed or currently serving as shelters, have transitioned to online learning delivered by TVET teachers. UNICEF is also supporting the identification of Temporary Learning Spaces (TLS) to enable online and blended learning for displaced youth. Digital TVET modules are being prepared for rollout in selected shelters, with additional modules under development to further expand access.

In parallel, 15 TVET schools currently functioning as shelters and hosting 8,161 IDPs have received in-kind support from UNICEF, valued at US\$ 50,000, to cover urgent needs including basic supplies, fuel, and essential materials. Non-Formal skilling programmes continue in non-affected areas, reaching 610 youth, while delivery in affected locations has shifted to online modalities where feasible.

Youth are also directly contributing to emergency response efforts through practical service delivery. Youth-produced items have been distributed across 26 TVET shelters, including 682 sleeping bags, 500 blankets, 200 Eid clothing kits, and 3,119 winter kits. Under work-based learning and income-generation initiatives, 41 youth supported food security efforts by producing 19,871 hot meals in coordination with WFP across three TVET shelter kitchens, while 71 youth in Baalbek produced and distributed 4,106 cold meals across 26 shelters. An additional 22 youth have initiated sleeping bag production, with 300 completed to date. Six youth barbers have also contributed to hygiene support, providing haircuts to 65 IDPs in two shelters.

Youth engagement in social cohesion and psychosocial support activities is ongoing, with 21 trained youth facilitators delivering sessions and the adolescent kit, reaching 461 children and adolescents across six TVET shelters.

Risk Communication and Community Engagement, Accountability to Affected Populations, Prevention of Sexual Exploitation and Abuse, Gender, and Inclusion

Between 26 and 31 March, UNICEF call centres received 1,344 calls from IDPs and host communities. Inquiries primarily related to shelter access for non-Lebanese populations, emergency cash assistance, non-food items (NFIs), access to newly introduced online learning services, and WASH gaps in informal settlements in Akkar. All cases were followed up through referrals and appropriate actions to ensure timely response to community needs.

To better understand gaps among the most vulnerable, UNICEF partnered with the women-led organization FEMALE to conduct rapid assessments of families living in open spaces in Beirut and Saida. To date, 240 households have been assessed, with an average household size of 4.8 persons. Findings indicate that many households are unable to access shelters due to overcrowding, safety concerns, access restrictions, and transportation barriers. Critical gaps include limited access to sanitation, lack of privacy, protection risks, and lack of access to education for children. Referrals for high-risk cases are ongoing, particularly for pregnant and lactating women, young children, persons with disabilities, and separated children. UNICEF is responding through targeted interventions, including mobile medical units, preparation of tailored education responses, and advocacy to address access barriers.

UNICEF convened 179 frontline workers from implementing partners across governorates to strengthen safeguarding and Prevention of Sexual Exploitation and Abuse (PSEA) measures in the emergency response, with a focus on community awareness, reporting mechanisms, and ethical engagement with informal leaders during assistance delivery.

Efforts to strengthen disability inclusion are ongoing. UNICEF, in collaboration with Organizations of Persons with Disabilities and the Emergency Taskforce for People with Disabilities, is leading a national service mapping to improve access to specialized and inclusive services for children and persons with disabilities.

Gender-responsive programming continues to be mainstreamed across the emergency response and flagship programmes. Gender-transformative leadership activities for and with girls are being implemented across 80 shelters and 33 Multi-Functional Platforms (MFPs), including POWER4Girls emergency sessions, girls' groups, and community initiatives. To date, 784 displaced adolescent girls (660 in shelters and 124 outside shelters) have been reached with POWER4Girls sessions, equipping them with critical information and practical skills to mitigate risks, including GBV. A total of 103 facilitators have been trained to deliver these sessions.

UNICEF has activated Risk Communication and Community Engagement (RCCE) mechanisms to support the response, including a communication tree, an updated landing page, and 11 information, education, and communication (IEC) materials disseminated across shelters and communities. These materials cover key sectors, including education and access to online learning platforms (e.g., Madristi and Call and Learn), child protection during emergencies (including prevention of family separation and alternative care), mental health and psychosocial support (including coping with stress and available services), as well as public health messaging on hygiene practices, lice prevention, and access to primary health care services. Partners are integrating RCCE approaches through school clubs, parent-teacher associations, and community networks, with 1,214 individuals engaged in participatory activities. An online chatbot was also shared with 2,071 users through U-Report.

Capacity-building efforts for responders continue to focus on RCCE, Accountability to Affected Populations (AAP), and PSEA. A social listening dashboard is actively monitoring community tensions and misinformation, informing daily response adjustments, while rapid RCCE needs assessments are being integrated into the Emergency Rapid Needs Assessment (ERNA).

Palestinian Programme

Essential and life-saving services continue to reach 20,971 IDPs (5,400 families) across UNRWA shelters and host communities in Palestinian camps and gatherings, including populations experiencing fluid displacement.

Multisectoral services, including Makani programming, are being delivered through operational centres across camps and gatherings, reaching 7,621 children with integrated education, child protection, health, and nutrition services, alongside systematic identification and referral of vulnerable cases.

Child protection and psychosocial support remain central to the response. To date, 5,291 children have accessed non-specialized, community-based psychosocial support, providing safe spaces for expression and emotional relief, while 25 children have received case management and access to specialized services.

Preventive and awareness-raising activities reached 5,099 individuals through child protection and children's rights sessions, promoting positive coping strategies during the emergency. In parallel, health and nutrition awareness sessions reached 5,507 children and adolescents, supporting improved hygiene practices and healthier behaviours in emergency conditions.

Adolescents and youth continue to be engaged through targeted resilience and social cohesion interventions. A total of 264 adolescents participated in life skills and healthy lifestyle sessions, while 268 benefited from non-formal skills training. Additionally, 49 youth have actively contributed to community-led initiatives, supporting the delivery of learning and recreational activities for affected children and adolescents.

Education response efforts are ongoing, with 1,000 children having undergone learning needs assessments. Based on the results, children will be grouped by academic level to receive targeted support. Interventions focus on retention and learning support for children aged 6–14, alongside early childhood education activities for children aged 3–5, to ensure age-appropriate learning opportunities and continuity of education during the crisis.

Efforts to strengthen safe and accountable access to services include PSEA and AAP awareness sessions, reaching 9,000 individuals with information on available services and feedback mechanisms.

Supply and Logistics

Since the onset of the emergency, UNICEF has mobilized US\$ 4.9 million in procurement to support its multi-sectoral response in Lebanon. An airbridge between Copenhagen and Beirut, established with support from UNICEF Supply Division and partners including ECHO, the EU Humanitarian Air Bridge (EU HAB), and DSV Logistics, has enabled the rapid delivery of critical supplies. To date, two charter flights carrying 72 metric tons of medical and emergency supplies have arrived, with a third shipment of 20 metric tons currently in preparation.

In parallel, US\$4.8 million worth of prepositioned supplies, equivalent to 1,100 metric tons, have been dispatched during the first month of the response. This includes US\$1.9 million in emergency supplies, comprising 71,790 emergency kits, 16,470 winter clothing kits, and 420,000 bottles of water, as well as US\$ 2 million in medical and nutrition supplies, in addition to education and WASH materials. Distributions have been implemented through government and local partners, including the MoPH, MEHE, MoSA, and civil society organizations. Funding is urgently needed to replenish essential supplies for continued distribution to the most vulnerable populations.

In hard-to-reach areas, distributions continue through coordinated convoys. The replenishment of emergency stocks is underway through local procurement, with 58,431 kits, including baby kits, hygiene kits, and dignity kits, currently in production.

Additional service providers have been mobilized to support critical infrastructure and service continuity, including the installation of latrines and showers in shelters, fuel delivery to water establishments, and maintenance of water systems nationwide. Printing of emergency learning materials for 50,000 children is also ongoing.

To enhance operational safety and access, an additional armoured vehicle has been procured to support staff deployment to hard-to-reach and high-risk areas.

UNICEF is also supporting the MoPH logistics operations at the Karantina Central Medical Store, with the deployment of a logistician contributing to warehouse and inventory management, as well as emergency distributions. Additional manpower and transport capacity are being provided to reinforce day-to-day operations.

Humanitarian Leadership, Coordination and Strategy

On 28 February 2026, MoSA, designated by the Government as the sole authority to coordinate with the Humanitarian Country Team (HCT) in the event of displacement, convened a meeting with Heads of UN Agencies to align on preparedness measures in anticipation of a potential escalation. During the meeting, MoSA confirmed that the agreed coordination architecture falls under the Lebanese Response Plan (LRP), through the Inter-Sector Coordination Group (ISCG) framework, led jointly by MoSA and the Ministry of Interior and Municipalities (MoIM). At the sub-national level, coordination is led by Governors in coordination with the National Disaster Risk Management Unit.

Within this framework, UNICEF co-leads the Education sector and leads the WASH and Nutrition sectors, as well as the Child Protection Working Group, at both national and sub-national levels. UNICEF also contributes to the Health, GBV and AAP, Gender, Livelihoods, and Basic Assistance sectors. UNICEF is also the co-lead of the Assessment and Analysis Working Group. At the sub-national level, the Operational Coordination Group (OCG) is coordinating the response in close collaboration with MoSA and Governors. UNICEF-led sectors are continuously monitoring partners' emergency response capacities and contingency stock levels, while ensuring that sector-specific preparedness actions are undertaken in coordination with partners.

UNICEF is delivering humanitarian programming for people in need across Lebanon in collective shelters and communities hosting IDPs; in hot spot areas such as directly conflict affected and hard-to-reach areas South of the Litani River and in the Bekaa; and in the border areas where people are seeking to leave Lebanon. This humanitarian response will continue to reach all vulnerable population groups including Lebanese, Syrians and Palestine refugees, including through leveraging funding from the LRP. UNICEF will deliver programmes that reach broad catchment populations such as operations and maintenance of the water systems and primary health care, while also supporting targeted interventions such as cash transfers to families and individuals and youth engagement.

Funding Overview and Partnerships

UNICEF's three-month Preparedness and Response Plan requires US\$ 48.3 million to reach 1 million people in need. There is US\$ 10.9 million in available funds for this response, in addition to an internal Emergency Programme Fund (EPF) loan of US\$ 6 million, an internal UNICEF financial mechanism that helps ensure readiness and rapid response during emergencies which will need to be reimbursed once funding is received. Despite this support, a 77 per cent funding gap remains. Without additional resources, UNICEF will be unable to maintain critical services or respond to the escalating needs of affected children and their families.

Funding Needs	Funds Available	Funding Gap
\$48.3m	\$10.9m	\$37.6m (77%)

Human Interest Stories and External Media

[CNN: Interview with Marcoluigi Corsi on the latest situation developments in Lebanon.](#)

[Al Sharq: Interview with Christophe Boulierac on the situation of children.](#)

[20 Minutes](#) and [Swiss Natcom Podcast](#): Interview with Andrea Berther on the situation of children and needed support.

[AFP: Interview with Atif Rafiq on children's education continuity and UNICEF support.](#)

[RND \(Germany\): Field visit with UNICEF and interview with Blanche Baz on the situation and support offered by UNICEF.](#)

[UNICEF Lebanon Press Release](#)

[France 24: Interview with Christophe Boulierac on the situation of children.](#)

[ABC 24: Interview with Marcoluigi Corsi on the latest situation developments in Lebanon.](#)

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